

THE ATHENAONE WORKFLOW OPTIMIZATION PLAYBOOK

A Quality Operations Guide
for FQHCs

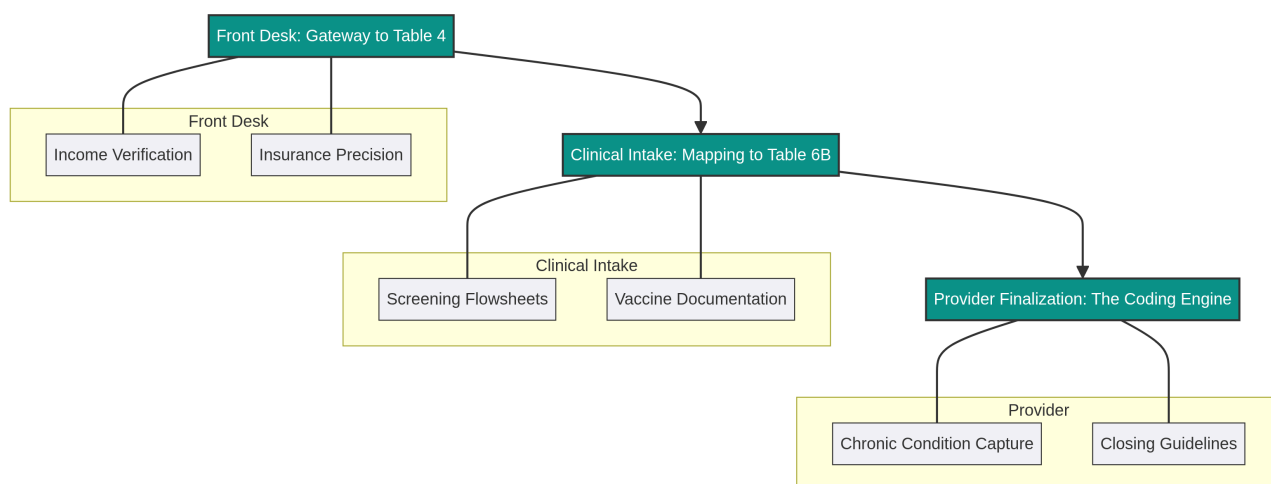
The AthenaOne Workflow Optimization Playbook: An FQHC Quality Operations Guide

Executive Summary

In the high-stakes environment of a Federally Qualified Health Center (FQHC), the Electronic Health Record (EHR) serves as the heartbeat of clinical operations, financial sustainability, and regulatory compliance. The 2025 Uniform Data System (UDS) reporting requirements have introduced more granular clinical quality measures (CQMs) and a heightened focus on electronic clinical quality measure (eCQM) accuracy. This expanded playbook moves beyond high-level strategy to provide specific, technical guidance for AthenaOne® optimization. By aligning daily clinical clicks with the technical specifications of the 2025 UDS Manual and leveraging the latest AthenaOne® clinical guidelines, health centers can ensure that every encounter is a compliant encounter.

1. Technical Workflow Optimization: The “Perfect” Encounter

A standardized visit workflow is the foundation of audit-readiness. Every step in the encounter must be mapped to a specific UDS table or CQM numerator/denominator requirement. This section details the critical actions required at each stage of the patient visit to ensure data integrity and compliance.



1.1 Front Desk: The Gateway to Table 4

The front desk is responsible for more than just scheduling; they are the primary data collectors for UDS Table 4 (Selected Patient Characteristics). Accuracy at this stage prevents significant reconciliation issues during the February reporting cycle.

1.1.1 Income Verification Best Practices

Ensure staff are capturing income as a percentage of the federal poverty guideline. This is not just for sliding fee scales; it is a critical demographic for Table 4, Lines 1-6. Implement standardized scripts to assist front desk staff in gathering this sensitive information accurately and respectfully.

Front Desk Script for Income Verification:

"To ensure we are providing you with the most accurate billing and to comply with our federal funding requirements, I need to update your household income information. This helps us determine if you qualify for any of our sliding fee discount programs. Could you please verify your current household size and estimated annual income?"

1.1.2 Insurance Precision and Reconciliation

Distinguish precisely between Medicaid (Line 8a) and CHIP Medicaid (Line 8b). Incorrect mapping here leads to massive reconciliation issues. Front desk staff must be trained to identify the specific plan types and enter them into the correct AthenaOne® insurance packages. Regular audits of insurance entry accuracy should be conducted to catch errors before they impact UDS reporting.

1.2 Clinical Intake: Mapping to Table 6B

The LPN or MA intake is where the majority of Table 6B (Quality of Care Measures) numerator data is captured. Structured data entry is paramount; narrative documentation will not suffice for automated reporting.

1.2.1 Screening Flowsheets and Structured Documentation

Screenings for tobacco use, depression, and BMI must be documented in the structured flowsheets of AthenaOne®. Narrative documentation in the “HPI” or “Assessment & Plan” sections will not be captured by automated reporting tools like Azara DRVS, leading to “data shadows” where care is provided but not credited.

Intake Checklist for Structured Data:

Screening Type	AthenaOne® Location	Required Action
Depression (PHQ-2/9)	Quality Management Tab / Flowsheets	Enter score; if positive, ensure follow-up plan is documented.
Tobacco Use	Social History / Flowsheets	Document status; if positive, initiate cessation counseling protocol.
BMI / Vitals	Vitals Section	Enter height and weight; ensure BMI auto-calculates.

1.2.2 Vaccine Documentation and 2025 Updates

With the 2025 updates to Pneumococcal (PVC21) and RSV guidelines, intake staff must review the patient’s age and risk factors immediately. RSV vaccination is now recommended for all adults 75+ and those 50-74 with specific comorbidities. Staff should utilize the “Historical Vaccine” tool with specific CVX codes to ensure accurate capture of immunizations administered outside the FQHC.

1.3 Provider Finalization: The Coding Engine

Providers must finalize encounters with an eye toward Table 6A (Selected Diagnoses and Services). Accurate coding directly impacts Risk Adjustment Factors (RAF) and ensures patients are correctly included in the denominators for key measures.

1.3.1 Chronic Condition Capture and RAF Optimization

Use precise ICD-10 codes for every chronic condition addressed during the encounter. This directly impacts RAF scores and ensures the patient is correctly included in the denominators for measures like Hypertension (CMS165) and Diabetes (CMS122). Providers should avoid unspecified codes whenever possible, opting for the highest level of specificity supported by the clinical documentation.

1.3.2 Closing the Guideline: Quality Management Tab Utilization

Providers should utilize the “Quality Management” tab in AthenaOne® to ensure all clinical guidelines are marked as “satisfied” or “excluded” before signing the note. This proactive step prevents care gaps from remaining open and ensures that the FQHC receives credit for the quality care provided.

2. Advanced Clinical Standardization

2.1 The “Exclusion” Strategy for Denominator Management

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2. Advanced Clinical Standardization

Clinical standardization is crucial for consistent quality outcomes and accurate UDS reporting. This section delves into advanced strategies for managing clinical workflows within AthenaOne® to optimize data capture and ensure compliance.

2.1 The “Exclusion” Strategy for Denominator Management

One of the biggest drivers of poor quality scores is “denominator inflation”—including patients in a measure who should be excluded. This is particularly relevant for conditions like Hospice & Frailty. The 2025 guidelines have updated exclusions for advanced illness and frailty. Ensure providers are using the correct AthenaOne® “Problem List” diagnoses (e.g., “Encounter for palliative care”) to automatically trigger these exclusions. For example, in Colorectal Cancer (CMS130), ensure exclusions for total proctocolectomy or colorectal cancer are documented in the Surgical History or Problem List to remove patients from the 45-75 age denominator.

Key Exclusion Documentation Points:

- **Hospice/Palliative Care:** Document in Problem List using specific ICD-10 codes (e.g., Z51.5 for Palliative Care). This should trigger an automatic exclusion from relevant quality measures.
- **Advanced Illness/Frailty:** Utilize the AthenaOne® Frailty Index or similar structured fields to capture and document these conditions, ensuring appropriate exclusions are applied.
- **Procedure-Based Exclusions:** For measures like CMS130 (Colorectal Cancer Screening), ensure that procedures such as total proctocolectomy are accurately recorded in the patient’s Surgical History within AthenaOne®.

2.2 TextMacro & OrderSet Technical Specifications

Standardization requires technical precision. AthenaOne® TextMacros and OrderSets are powerful tools for streamlining documentation and ensuring consistent data capture, but they must be configured correctly.

2.2.1 LOINC and SNOMED Mapping for TextMacros

Every macro used for a screening (e.g., “Negative for depression”) must be linked to the corresponding LOINC (Logical Observation Identifiers Names and Codes) and SNOMED (Systematized Nomenclature of Medicine) codes in the background. If you are building custom macros, work with your IT team to ensure they are “mappable” to these standardized terminologies. This ensures that data captured via macros is interpretable by external reporting systems like Azara DRVS and meets UDS specifications.

Example: Depression Screening TextMacro Configuration

TextMacro Phrase	Associated LOINC Code	Associated SNOMED CT Code	Purpose
“PHQ-9 score of 5, mild depression”	44261-6 (PHQ-9 score)	400190003 (Mild depression)	Standardize depression screening results for reporting.
“Patient denies suicidal ideation”	75900-7 (Suicidal ideation)	162380003 (No suicidal ideation)	Ensure critical safety information is consistently documented.

2.2.2 CPT-II Codes and OrderSet Configuration

Configure OrderSets to include the specific CPT-II codes that act as the “numerator” for quality measures. For example, for Diabetes (CMS122), an OrderSet for a diabetic visit should include the CPT-II code for A1c < 7.0% (e.g., 3044F). This automates the closure of care gaps without requiring extra clicks. Regularly review and update OrderSets to reflect the latest UDS and quality measure requirements.

Best Practices for OrderSet Configuration:

- **Automate CPT-II Capture:** Embed relevant CPT-II codes directly into OrderSets for common clinical scenarios (e.g., well-child visits, diabetic follow-ups, hypertension management).

- **Link to Quality Measures:** Ensure each CPT-II code within an OrderSet directly corresponds to a specific quality measure numerator, facilitating automated reporting.
- **Regular Review:** Establish a quarterly or semi-annual review process for all active OrderSets to ensure they remain aligned with current clinical guidelines and UDS requirements.
- **Provider Training:** Provide ongoing training to providers on the correct and efficient use of OrderSets to maximize their benefit in quality data capture.

3. Data Hygiene: Athena to Azara Reconciliation

The seamless flow of data from AthenaOne® to population health management tools like Azara DRVS is critical for accurate UDS reporting and effective quality improvement. This section identifies common integration vulnerabilities and provides actionable fixes.

3.1 Integration Vulnerability Matrix and Fixes

Below is an expanded “Integration Data Vulnerability Matrix” detailing common technical pitfalls in FQHC data pipelines and their corresponding solutions.

Integration Vulnerability	Technical Root Cause	2025 UDS Impact	Required Fix
Denominator Mismatch	Custom visit types in Athena not mapped to “Medical” in Azara.	Deflated patient counts on Table 5.	Map every Athena “Appointment Type” to the corresponding Azara “Visit Class.”
Missing Immunizations	Historical vaccines entered as “free text” rather than structured data.	Failed Childhood Immunization (CMS117) rates.	Train staff to use the “Historical Vaccine” tool with specific CVX codes.
Lab Result Lag	Inbound labs (e.g., A1c, Pap) not “resulted” to the correct flowsheet.	Open care gaps on Table 7 (Health Outcomes).	Audit the “Lab Interface Mapping” to ensure vendor results land in structured fields.
Problem List Inaccuracy	Inconsistent use of ICD-10 codes for chronic conditions.	Inaccurate RAF scores and measure denominators.	Standardize problem list entry protocols; provide regular coding education.
Referral Tracking Gaps	Referrals documented in notes but not in structured referral module.	Failed Closed-Loop Referral (CLR) measures.	Mandate use of AthenaOne® referral module for all outgoing referrals.

3.1.1 Denominator Mismatch Resolution

Ensure every custom AthenaOne® “Appointment Type” is meticulously mapped to the corresponding Azara “Visit Class.” This prevents patient encounters from being inadvertently excluded from population health dashboards and UDS Table 5 denominator counts. Regularly review appointment type configurations in both systems.

3.1.2 Missing Immunizations Data Integrity

Historical immunizations are often entered as free text, rendering them invisible to automated reporting. Staff must be rigorously trained to use the “Historical Vaccine” tool within AthenaOne® and select the appropriate CVX codes. This structured data entry is essential for accurate Childhood Immunization Status (CMS117) rates.

3.1.3 Lab Result Lag Optimization

Inbound lab results (e.g., A1c, Pap smears) must be configured to “result” directly into the correct structured flowsheets within AthenaOne®. When lab results are not mapped correctly, they create open care gaps on UDS Table 7 (Health Outcomes). Conduct quarterly audits of lab interface mapping to ensure all critical lab results populate the appropriate structured fields for automated measure calculation.

4. 2025 UDS Measure Deep Dive

The 2025 UDS Manual introduces specific nuances for various clinical quality measures. A deep understanding of these technical specifications is paramount for accurate reporting and maximizing FQHC performance. This section provides an in-depth look at key UDS measures and strategies for optimization within AthenaOne®.

4.1 Childhood Immunization Status (CMS117v13) Optimization

This measure is notoriously difficult to optimize. To be successful, the patient must have the full series completed by their second birthday. The Workflow: Use the “Huddle” tool to identify 18-month-olds who are missing their 4th DTaP or 3rd IPV. This is your “last chance” window for compliance. Proactive outreach and efficient scheduling are critical.

Strategies for CMS117v13 Improvement:

- **Huddle Integration:** Integrate CMS117v13 status into daily huddles. Identify all 18-month-old patients with upcoming appointments who are not up-to-date on immunizations.
- **Pre-Visit Planning:** For identified patients, pre-charting should include a review of their immunization history and a plan to administer missing vaccines during the visit.
- **Patient Education:** Provide clear, concise patient education materials on the importance of timely immunizations and the UDS requirements.
- **Recall System:** Implement a robust recall system for patients who miss their immunization appointments, prioritizing those approaching their second birthday.

4.2 Cervical Cancer Screening (CMS124v13) Optimization

The Technicality: The 2025 manual emphasizes that “self-collected” samples for HPV testing are now acceptable in specific scenarios. Ensure your AthenaOne® lab orders distinguish between provider-collected and self-collected to ensure proper credit. This expands access and improves screening rates, particularly for hard-to-reach populations.

Key Considerations for CMS124v13:

- **Self-Collected HPV:** Develop clear workflows for offering and documenting self-collected HPV samples. Ensure AthenaOne® lab orders are configured to accurately reflect the collection method.
- **Patient Eligibility:** Educate providers and staff on the specific criteria for self-collected HPV testing as per the 2025 UDS Manual.
- **Follow-up Protocols:** Establish robust follow-up protocols for abnormal self-collected HPV results, ensuring patients receive timely diagnostic and treatment services.
- **Patient Outreach:** Utilize AthenaOne® patient portal and messaging features for targeted outreach to eligible patients due for cervical cancer screening, including information on self-collection options.

4.3 Diabetes: Hemoglobin A1c Poor Control (CMS122v13) Optimization

This measure focuses on patients with diabetes whose most recent HbA1c is > 9.0%. Effective management requires a multi-faceted approach, including patient education, medication adherence, and lifestyle interventions.

Strategies for CMS122v13 Improvement:

- **Registry Management:** Utilize AthenaOne®’s patient registry functionality to identify patients with poorly controlled diabetes. Prioritize these patients for proactive outreach and intensive management.
- **Care Team Collaboration:** Implement a team-based approach involving providers, nurses, dietitians, and educators to support patients in achieving glycemic control.
- **Medication Adherence:** Leverage AthenaOne®’s e-prescribing and medication reconciliation features to improve medication adherence. Consider integrating

pharmacy services for medication therapy management.

- **Patient Education:** Provide comprehensive education on diabetes self-management, including diet, exercise, blood glucose monitoring, and the importance of regular follow-up.

4.4 Hypertension: Blood Pressure Control (CMS165v13) Optimization

This measure assesses the percentage of patients aged 18-85 years who had a diagnosis of hypertension and whose blood pressure was adequately controlled ($<^{140}_{/90}$ mmHg) during the measurement period. Accurate and consistent blood pressure measurement is fundamental.

Strategies for CMS165v13 Improvement:

- **Standardized BP Measurement:** Ensure all clinical staff are trained and adhere to standardized protocols for blood pressure measurement, including proper cuff size, patient positioning, and technique.
- **Home Blood Pressure Monitoring (HBPM):** Encourage and educate patients on HBPM. Integrate HBPM data into AthenaOne® to inform clinical decision-making and track progress.
- **Medication Management:** Optimize antihypertensive medication regimens based on individual patient needs and guidelines. Utilize AthenaOne®'s clinical decision support tools to guide prescribing.
- **Lifestyle Interventions:** Provide counseling and resources for lifestyle modifications, including dietary changes (e.g., DASH diet), regular physical activity, and weight management.

5. Audit Readiness: The “OSV” Checklist

The HRSA Operational Site Visit (OSV) is the ultimate test of your workflows and documentation. Proactive preparation, guided by a comprehensive checklist, is essential for a successful audit. This section outlines key areas of focus for OSV readiness within the context of AthenaOne® workflows.

5.1 Credentialing & Privileging: EHR Clinical Case Routing

Does your EHR (AthenaOne®) support “Clinical Case Routing” to staff with the appropriate licensure? HRSA auditors will verify that only appropriately credentialed and privileged staff are performing tasks within their scope of practice. Ensure that AthenaOne® user roles and permissions are meticulously configured to reflect staff licensure and responsibilities. This includes clear documentation of who can sign off on specific types of encounters or orders.

OSV Checklist Item: Credentialing & Privileging

- **AthenaOne® User Roles:** Verify that all AthenaOne® user roles align with staff licensure and scope of practice.
- **Clinical Case Routing:** Demonstrate how AthenaOne® routes clinical tasks (e.g., medication refills, lab result review) to appropriately licensed personnel.
- **Documentation of Competency:** Ensure there is a clear process for documenting and tracking staff competency for tasks performed within AthenaOne®.

5.2 Clinical Oversight: LPN-led Intake Review

Can you produce a report showing that every LPN-led intake was reviewed and signed by a licensed provider? This is a common area of scrutiny during OSVs. AthenaOne® should be configured to facilitate and track provider co-signature or review of LPN/MA documentation. This demonstrates appropriate clinical oversight and ensures the accuracy and completeness of the patient record.

OSV Checklist Item: Clinical Oversight

- **Provider Co-signature Workflow:** Demonstrate the AthenaOne® workflow for provider review and co-signature of LPN/MA documentation.
- **Audit Trail:** Be prepared to generate reports from AthenaOne® showing a clear audit trail of provider review for LPN-led intakes.
- **Policy & Procedure:** Ensure your FQHC has a clear policy outlining the requirements for clinical oversight of LPN/MA documentation.

5.3 Closed-Loop Referrals: Tracking and Completion

Does your “Referral Module” show a “Completed” status for at least 80% of specialty requests? HRSA expects FQHCs to demonstrate effective closed-loop referral processes. AthenaOne®’s referral management module should be fully utilized to track referrals from initiation to completion, including documentation of specialist visits and receipt of consultation reports. This ensures continuity of care and prevents patients from falling through the cracks.

OSV Checklist Item: Closed-Loop Referrals

- **Referral Tracking:** Demonstrate the use of AthenaOne®’s referral module to track all outgoing referrals.
- **Completion Rates:** Be able to report on the percentage of referrals that reach a “Completed” status, including documentation of specialist appointments and consultation notes.
- **Follow-up Process:** Outline the process for following up on outstanding referrals and addressing barriers to completion.

Conclusion

Moving from “fluff” to “function” requires a relentless focus on data integrity and workflow optimization. This playbook provides the technical blueprint to transform AthenaOne® into a high-performance quality engine. By mastering the technicalities of the 2025 UDS Manual and the clinical logic of AthenaOne®, your health center will not only survive the reporting season—it will lead the way in population health. The strategies outlined herein are designed to ensure audit readiness, enhance data accuracy, and ultimately improve patient outcomes within the FQHC setting.

References

References

[1] Uniform Data System (UDS) Manual, 2025. Health Resources and Services Administration (HRSA). [2] AthenaOne® Clinical Guidelines and Documentation Best

Practices. [3] Azara DRVS User Manual and Integration Guide.